

Appendix A: Vermont System Portrait

Demographics, Hospitals, Insurance, Workforce, and System in Motion

How to use this appendix

This appendix consolidates the Vermont-specific factual foundation for the book's national argument. Chapter 2 embeds only the fiscal crisis data needed to understand why sequencing matters in Vermont; this appendix provides the complete system portrait that Chapter 2 and subsequent chapters draw on. References throughout the book to Vermont's demographics, hospital system, regulatory architecture, and workforce data point here. Each section is self-contained and can be read independently.

Former Chapter 2 content (Vermont System Portrait) has been fully integrated here, supplemented by the January 2026 hospital transformation plan data from AHS tracking records and the GMCB regulatory architecture from ch2_additions.

A.1 Vermont: The State

Vermont is the second-smallest state by population in the United States. Its 647,000 residents live across 14 counties and 255 towns and cities in a predominantly rural landscape — 9,616 square miles of mountains, valleys, farms, and forest. The population density is 70 people per square mile, well below the national average of 94. Nearly two-thirds of Vermont residents live in areas classified as rural by the federal definition used for healthcare program eligibility.

Vermont's geography is not merely a demographic fact. It is a healthcare infrastructure constraint. A patient in Essex County — one of the most rural and least populated counties in the Northeast Kingdom, with an uninsurance rate of 8% compared to Vermont's statewide 3% — may be 45 minutes from the nearest hospital emergency department, 90 minutes from a specialist, and hours from a tertiary care center. Public transportation is limited or unreliable in most rural counties. Broadband penetration reaches 80% of rural areas, leaving 20% without the connectivity that telehealth and remote patient monitoring require.

Vermont's economy is diverse but small. Healthcare is one of the most common industries in the state — employing approximately 14% of the workforce and representing a major economic anchor in rural communities where the hospital is often the largest employer. The University of Vermont Medical Center in Burlington, with approximately 50% of the state's hospital market share, is the economic center of gravity of Vermont's healthcare system.

Every reform that affects UVMMC affects the Vermont economy in ways that hospitals in larger, more economically diverse states do not.

A.2 Vermont Demographics and Population Projections

Vermont's demographic trajectory is the foundational fact of its healthcare transformation challenge. Oliver Wyman's population projections, derived from MPR Vermont population data, tell the story with precision.

Indicator	Value	Significance
Vermont population aged 65+ by 2040	30%+	Up from 21.7% in 2020 — a 57% increase in the elderly population
Working-age population change by 2040	−13%	From 367,000 to approximately 318,000 — the commercial insurance base shrinks
Total Vermont population 2020–2040	Flat	Approximately 612,000–624,000 throughout — aging, not growing
Vermont residents with health insurance (2025)	97%	One of the highest coverage rates in the country — ACA and Medicaid expansion impact
Primary care provider shortage projected by 2030	370 FTE	112 in family medicine, 190 in other primary care specialties (RHT Program application)
Rental vacancy rate	3%	Below healthy 5% threshold — housing as healthcare workforce constraint

Figure A.1 — Vermont demographic transformation: key projections and their healthcare implications.

Source: Oliver Wyman Act 167 Report (2024); Vermont RHT Program Application; KFF.

The implications of these demographic trends are not complicated — they are simply severe. An aging population means a shift in the payer mix toward Medicare and Medicaid, both of which reimburse hospitals below cost. A shrinking working-age population means a shrinking commercially insured population, reducing the cross-subsidy that currently sustains hospital operations. More complex care needs among the elderly means higher costs per patient. And an aging healthcare workforce — Vermont has the same age distribution among its clinicians as among its population generally — means accelerating retirements of experienced providers in exactly the years when demand is rising.

Oliver Wyman's analysis named this dynamic directly: the cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain as demographics shift. The commercial premiums that make it possible for hospitals to accept below-cost Medicare and Medicaid payments depend on a sufficient pool of commercially insured patients. As that pool shrinks relative to the Medicare and Medicaid population, the

math stops working — not in a distant future scenario, but on the trajectory Vermont is already on.

A.3 Vermont's Regulatory Architecture: The Green Mountain Care Board

Vermont's healthcare transformation would be impossible without an institutional actor that most other states do not have. The Green Mountain Care Board — a five-member independent state body created in 2011 — is the regulatory engine that converts Vermont's legislative mandates into binding operational obligations for its hospital system. Understanding the GMCB is prerequisite to understanding why Vermont can attempt what other states cannot.

Vermont's hospital budgets have been subject to state review since 1983, but GMCB's assumption of regulatory authority beginning in fiscal year 2013 qualitatively changed the nature of that oversight. The GMCB does not merely review hospital budgets — it establishes them. Under Vermont statute, each hospital must operate within the budget the Board sets. When hospitals exceed those budgets, the GMCB can enforce by reducing future commercial rate approvals. When hospitals face financial distress, the GMCB Chair meets directly with hospital leadership. When the Legislature mandates transformation planning under Acts 167 and 68, GMCB is the institutional receptor for the resulting analysis and the body responsible for translating policy architecture into hospital-level operational requirements.

The practical significance of this regulatory architecture extends beyond Vermont's 14 hospitals. GMCB's annual budget review process, conducted entirely in public meetings under Vermont's Open Meeting Law with mandatory participation from the Office of the Health Care Advocate, has produced a documentary record of Vermont's hospital system that is unmatched in American health policy. For FY25, GMCB approved \$3.7 billion in system-wide hospital spending — a 4.1% increase after hospitals collectively requested 8%. The Board also enforced FY23 budget overages against UVMHC (\$80.3M) and RRMC (\$11.1M), reducing future commercial rate approvals as a consequence.

GMCB function	What it does and why it matters for transformation
Hospital budget review	Annually establishes Net Patient Revenue (NPR) growth limits for all 14 hospitals. For FY25, GMCB approved \$3.7 billion in hospital spending — a 4.1% increase after hospitals requested 8%. This regulatory compression drives hospital attention to efficiency and alternative revenue models.
Budget enforcement	When hospitals exceed approved budgets, GMCB can enforce by reducing future commercial rate approvals. In FY23, UVMHC exceeded its budget by \$80.3M and RRMC by \$11.1M — GMCB enforced both. This enforcement authority distinguishes Vermont's regulatory model from purely advisory state oversight.

Monthly performance monitoring	All 14 hospitals submit monthly financial performance reports throughout the year. This continuous monitoring capability is the operational substrate for Vermont's transformation accountability.
Reference-based pricing (RBP)	Act 68 mandates RBP beginning FY2027, with GMCB setting the methodology. RBP will cap commercial hospital rates as a percentage of Medicare. GMCB's existing rate review infrastructure makes mandatory RBP implementable.
Global budget design	GMCB will set prospective global hospital budgets beginning FY2028–2030, in coordination with the AHEAD Model. The analytics vendor procurement (McKinsey, 2025–2026) is GMCB building the technical infrastructure for global budget methodology.
Hospital sustainability planning	Under Act 167, GMCB commissioned the Oliver Wyman analysis and is the institutional receptor for its recommendations. GMCB's public meeting record is the documentary foundation for Vermont's transformation case.

Figure A.2 — GMCB regulatory functions and their significance for Vermont's transformation agenda.
Source: GMCB Hospital Budget Review Guide; Acts 167 and 68; GMCB Annual Report 2024.

GMCB's institutional design explains a specific feature of Vermont's transformation that is analytically important: mandatory authority precedes payment reform deployment. The voluntary ACO era produced modest results because the highest-cost providers could decline to participate. GMCB's mandatory budget authority — its ability to set rates rather than merely recommend them — is the structural condition that makes Vermont's mandatory reference-based pricing and global budgeting achievable. Other states attempting payment reform without a comparable regulatory body are, in the framework's terms, attempting Economics pillar transformation without the Policy pillar architecture that gives payment reform enforcement teeth.

A.4 Vermont's Hospital System

Vermont has 14 general acute care hospitals, all of which are non-profit. This fact — no for-profit hospitals — is significant for the reform agenda: Vermont's hospital system does not have the shareholder value obligations that complicate transformation discussions in states with investor-owned hospital chains. Vermont's hospitals are community assets, governed by community boards, with legal missions oriented toward community health. This creates better conditions for the kind of collaborative transformation Vermont is attempting — but it does not eliminate the financial pressures, institutional interests, or organizational inertia that make transformation difficult everywhere.

Of Vermont's 14 hospitals, the University of Vermont Health Network comprises UVMMC, CVMC, and Porter Hospital as system hospitals. Dartmouth Health affiliates include SVMC and Mt. Ascutney. The remaining hospitals — Gifford, Copley, NCH, NVRH, BMH, Springfield, RRCM, Grace Cottage, and NMC — operate as independent community institutions or loose collaborative members, and it is these independent hospitals that face the most acute financial fragility.

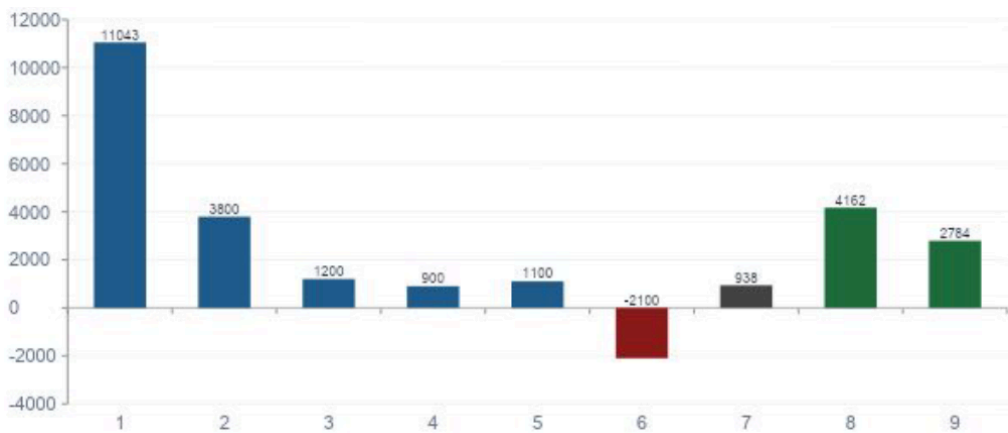


Figure A.3 — Vermont Hospital Financial Positions: FY2023 vs. national benchmarks. Source: Claude_HTR_Visual_Companion_Deck.pptx, Slide 10. AHS Health Care System Transformation Report (November 2025); NASHP via CMS.

The distribution of financial distress across hospital categories matters for reform design. Critical Access Hospitals — the smallest, most rural hospitals serving the communities with the fewest alternatives — are the most financially distressed, with an average operating profit of \$938 per adjusted discharge versus a \$2,784 benchmark. They are also the hospitals where service reductions or closures would have the most severe community consequences, and the hospitals that the AHEAD Model's global budget design must protect most carefully. UVMMC's administrative cost per adjusted discharge was \$3,826 against a national benchmark of \$1,427 — structural cost problems that voluntary reform cannot address.

The Fourteen Hospitals: Transformation Status and Challenges (January 2026)

The January 2026 transformation plans submitted to AHS under Act 68 provide a uniquely granular view of where each hospital stands. These are operational documents produced in response to a statutory mandate, reviewed by AHS staff, and directly tied to Rural Health Transformation Program grant funding. The following table characterizes all 14 hospitals by type, transformation plan grant status, primary focus, and key challenges as documented in the January 2026 submissions.

Hospital	Type	Grant	Primary transformation focus	Key challenges / concerns
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UVM Medical Center (UVMHC)	Academic hub	Not eligible	Technology solutions for acuity management and transfers; statewide reference hub	Administrative cost \$3,826/adj. discharge vs. \$1,427 benchmark; FY23 budget overage enforcement (\$80.3M)
Central Vermont Medical Center (CVMC)	Regional PPS	Awarded	Technology solutions for acuity management and patient transfers	Part of UVMHCN system; subject to system-level budget constraints
Porter Hospital	Regional PPS	Awarded	Technology solutions for acuity management and patient transfers (shared strategy with CVMC)	Part of UVMHCN system; FY23 budget overage of \$11M
Rutland Regional Medical Center (RRMC)	Regional PPS	Awarded	Hospice utilization improvement; dermatology in community; regional collaboration with SVMC	FY23 budget overage of \$11.1M (partially enforced); exploring regional service lines with SVMC
SW Vermont Medical Center (SVMC)	Regional PPS	Awarded	RN Bridge to PCP program; \$29M cancer center; DaVinci robot; Epic EHR migration (\$30M); adolescent mental health unit	Locum provider costs; care coordination gaps; transport barriers for non-acute patients
Brattleboro Memorial Hospital (BMH)	Rural PPS	Awarded	Mobile Integrated Health (MIH) program; FQHC Look-Alike feasibility; birthing and oncology sustainability	GMCH FY26 deep concern about solvency; operating losses FY22/FY24/projected FY25; executive bonus scrutiny
Springfield Hospital	Rural PPS	Needs follow-up	Clinical services redesign feasibility study; relationship with Northstar Health	Oncology and cardiology sustainability concerns; plan not approved as of Jan 2026
Gifford Medical Center	CAH + FQHC	Awarded	Hospital-to-hospital transfer program with UVMHC/DHMC; NECHN GPO (\$793K projected)	Targeting avg. daily census of 20 for sustainability; tele-radiology vendor capacity constraints

			savings); shared radiology	
Copley Hospital	CAH	Awarded	Consulting services; primary care access expansion	Plan quality concerns; GMCB required FY25 budget modifications; sustainability of changes under discussion
North Country Hospital (NCH)	CAH	Awarded	Regionalization planning with NVRH; shared project manager and legal support; pulmonology; tele-neonatology	Limited IT staff; dependent on Vermont CIN shared services model for analytics capability
Northeastern VT Regional Hospital (NVRH)	CAH	Awarded	Regionalization planning with NCH; cardiology; tele-ICU	FY23 budget overage (not enforced); GMCB monitoring; limited workforce pipeline in Northeast Kingdom
Mt. Ascutney Hospital (MAHHC)	CAH + FQHC	Awarded	Age-Friendly Program / geriatric COE; Collaborative Care Model via DHMC; shared CEO/CFO/COO with Valley Regional (\$600K savings)	EMR transition (Oct 2025) disrupted data; subsidizing pharmacy (\$50K/yr); residential care facility support (\$950K in FY25)
Grace Cottage Hospital	CAH	Awarded	\$24M primary care clinic capital campaign; same-day access clinic; swing bed program; AI ambient scribe (Coverys grant)	Denial reduction \$76K→\$1,274 (FY25 success); FY24 loss of \$4M reduced to ~\$1M in FY25; no DSH payments
Northwestern Medical Center (NMC)	CAH	Awarded	Innovative birthing services retention; shared cardiologist with Copley; oncology	Questions on value of marketing campaigns; schedule not yet confirmed as of Jan 2026

Figure A.4 — Vermont's 14 hospitals: typology, transformation grant status, primary focus, and key challenges. *Source: AHS Hospital Tracking data (January 2026); individual hospital transformation plans (January 15, 2026); GMCB FY26 Budget Orders.*

TIER 1	Regional Specialty Centers (RSC) UVMHC (16 COEs) SW Vermont Medical Brattleboro Memorial Rutland Regional Central VT Medical Highest COE count. Accept referrals from the network for designated specialties. Financial sustainability strongest. Primary RSC role: UVMHC.
TIER 2	Community Hospitals — Focused Scope NW Medical Center NE VT Regional Springfield Copley Mt. Ascutney Porter Medical 2–4 COE designations. Maintain specific service lines. Transfer complex cases to RSCs. Actively engaged in Act 68 transformation planning.
TIER 3	Transition Assessment Required Grace Cottage (19 beds) Gifford Medical North Country Hospital Most financially vulnerable. REH conversion, CACC, or home-based care model under assessment. Act 68 transformation plans address transition path.

Figure A.5 — Vermont's 14-Hospital Network: Three-Tier Structure (Tier 1 Regional Specialty Centers, Tier 2 Community Hospitals, Tier 3 Transition Assessment). Source: Claude_HTR_Visual_Companion_Deck.pptx, Slide 11. Oliver Wyman Act 167 Report (2024); AHS November 2025 Transformation Report.

Patterns in the Data

The administrative cost problem is concentrated at the top of the system. Oliver Wyman's finding that UVMHC's administrative cost per adjusted discharge was \$3,826 against a national benchmark of \$1,427 is not merely an efficiency observation. It is evidence that Vermont's largest and most financially powerful hospital has structural cost problems that voluntary reform cannot address. GMCB's enforcement of UVMHC's FY23 budget overage, and its decision to not approve NPR growth for BMH in FY26 while expressing deep concern about BMH's solvency, illustrate the regulatory tools available when transformation accountability is backed by statutory authority.

The regionalization logic is visible in the transformation plans but is not yet architecturally formed. NCH and NVRH are explicitly partnering to share a project manager, legal counsel, and subject matter expertise for regionalization planning. Gifford has built a functioning hospital-to-hospital transfer protocol with UVMHC and Dartmouth Health. Mt. Ascutney and Valley Regional Hospital have shared their CEO, CFO, and COO since late 2024, generating approximately \$600,000 annually in administrative savings. SVMC and RRMC are exploring shared specialty and support service lines. These are the early formations of the regional hub structure that Acts 167 and 68 envision, but they are bottom-up and incomplete rather than top-down and comprehensive.

The analytics gap is an operational reality, not a theoretical concern. The AHS Hospital Transformation Analytical Support RFP, issued in late 2025 and awarded to McKinsey, acknowledged that Vermont does not yet have the analytics infrastructure to manage global budgets, measure population health outcomes at the hospital level, or track patient migration across its 14-institution system. The scope of that procurement — VHCURES integration,

facility risk profiling, patient migration modeling, scenario modeling — reveals both the ambition of Vermont's transformation and the distance between current analytics capability and what AHEAD's launch in January 2027 will require.

A.5 The Transformation Plans: Vermont's System in Motion (January 2026)

The 148 individual transformation initiatives documented across Vermont's hospital plans in January 2026 are not policy proposals. They are operational commitments with named responsible parties, defined timelines, cost estimates, and measurement frameworks — produced under statutory obligation and reviewed by AHS staff who rate their quality and track their progress. This is the Operations pillar working in real time.

Initiative theme	Initiative count	Representative examples from hospital plans
Clinical service redesign	~55	SVMC RN Bridge to PCP; BMH MIH formalization; MAHHC Age-Friendly Program; Grace Cottage same-day access clinic; RPMC hospice utilization improvement
Regionalization / transfers	~20	NCH-NVRH shared regionalization planning; Gifford hospital-to-hospital transfers with UVMC/DHMC; MAHHC shared senior leadership with Valley Regional; SVMC-RPMC regional service lines (allergy, ENT, pain, ERCP)
Workforce development	~25	MAHHC Nursing Pathways Program (\$190K for two cohorts); SVMC 'grow our own' RN pipeline; Grace Cottage near-elimination of travel nurses; Mt. Ascutney SANE forensic capacity building
Technology and data	~20	SVMC Epic migration (\$30M); CVMC/Porter acuity management technology; Grace Cottage EMR interoperability interfaces; Grace Cottage ambient AI scribe (Coverys grant)
Shared purchasing / operational efficiency	~15	NECHN GPO launch (Gifford projected \$793K savings); captive insurance pool across NE Collaborative members; MAHHC-Valley Regional shared C-suite (\$600K annual savings)
Value-based care / payment alignment	~13	SVMC Belong Health Medicare Shared Savings ACO (2026); SVMC Medicaid global budget participation; Grace Cottage ACO participation and Vatica

		integration; Gifford FQHC model expansion
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Figure A.6 — Vermont hospital transformation initiative themes, January 2026. Based on 148 initiatives from AHS Transformation Plans Analysis. *Source: AHS Transformation_Plans_Analysis.xlsx (January 2026).*

Three case studies in transformation from primary sources

Grace Cottage Hospital reduced insurance denials from \$76,160 in FY2024 to \$1,274 in FY2025 — a 98% reduction — through concurrent review for acute admissions, documentation improvement, and provider education. Its financial trajectory — from a \$4 million pre-audit operating loss in FY24 to approximately \$1 million in FY25 to projected break-even in FY26 — with no DSH payments and a 5-star National Rural Health Star Rating, makes it the book's most important counterexample to the narrative that Vermont's small CAHs are uniformly failing.

Mt. Ascutney Hospital's Community Health Team data — 1,132 individuals served over ten months in 2025, 80% over age 64, 22% requiring six or more contacts — is the most concrete available illustration of Vermont's demographic aging problem at the clinical level. The CHT is not serving a theoretical future population; it is already serving a population whose overwhelming predominance is geriatric, complex, and high-utilizing.

Mt. Ascutney's shared senior leadership arrangement with Valley Regional Hospital — shared CEO since December 2024, shared CFO since November 2025, shared COO since January 2026 — generating \$600,000 annually in administrative savings, while simultaneously subsidizing Windsor's only pharmacy (\$50K/yr) after the town's private pharmacy closed in 2025. This is the equity problem that rural hospital transformation must address: a small CAH bearing community infrastructure costs that its operating revenue cannot sustainably support.

What the Plans Reveal About the System's Constraints

Interoperability remains a solved problem only on paper. Grace Cottage's plan documents the reality with precision: working with multiple regional partners who operate on different electronic medical records, "the process from order to completion is arduous and full of administrative waste" — relying on "antiquated and insecure technologies" because secure interoperability infrastructure does not yet connect Vermont's system. Vermont's HIE, VITL, exists but is not functionally integrated with the workflows of most community hospitals. This is not a technology problem awaiting a solution — the solutions exist. It is an incentive and governance problem: no single hospital can justify the cost of building interoperability infrastructure alone, but the collective benefit to the system is substantial. This is a textbook Technology-Operations dependency failure.

Community pharmacy access has emerged as a new access gap. Mt. Ascutney is subsidizing Windsor's only pharmacy after the town's private pharmacy closed in 2025. Grace Cottage's

pharmacy fills over 6,000 scripts monthly as a community service, collecting patients from Brattleboro who have lost local pharmacy access. When a hospital becomes a community's pharmacy, its cost structure reflects that community function in ways that standard efficiency benchmarks do not capture.

The evidence that voluntary participation cannot achieve structural change runs through the plans like a thread. BMH's MIH program explicitly identifies sustainable funding as its primary barrier, noting that standard billing cannot cover the cost and requesting that AHS provide reimbursement through RHTP funding. NVRH and NCH's regionalization work explicitly identifies legal counsel and project management as shared costs they cannot individually afford. These are not capability failures — they are financing failures. The hospitals know what needs to be done. The current financial architecture makes it impossible to fund. This is precisely the condition that Acts 167 and 68 are designed to address.

A.6 The Insurance and Premium Crisis

Vermont's health insurance market is dominated by Blue Cross Blue Shield of Vermont and MVP Health Care. Vermont was an early adopter of the ACA, expanding Medicaid in 2014 and establishing Vermont Health Connect, the state's insurance exchange. The result is a 97% coverage rate — one of the highest in the country — that is a genuine policy achievement.

The achievement of near-universal coverage has not, however, resolved Vermont's affordability crisis. Coverage is not the same as affordability. Vermont's individual market monthly premium rose from \$456 in 2018 to \$948 in 2024 — a 108% increase — while median household income grew 22%. Out-of-pocket maximums more than doubled. The insured Vermonter who delays necessary care because of the cost of their deductible or co-pay is not a healthcare access success story; they are an indictment of a system that has achieved coverage on paper while allowing costs to rise beyond reach.

The mechanism connecting hospital prices to insurance premiums is not complicated. Vermont hospitals charge commercial insurers 250–300% of Medicare rates on average for hospital services. In the Burlington area, UVMMC charged approximately 417% of Medicare for outpatient services in 2022. These prices — which bear no relationship to quality — translate directly into premiums. When a hospital charges 417% of Medicare for outpatient imaging, every insurer that covers that hospital's patients must charge premiums high enough to pay those prices and still maintain solvency. The result is the premium trajectory Vermont has experienced: driven primarily by hospital price increases that have no regulatory brake. This is what reference-based pricing is designed to address, and why Act 68's mandate to implement RBP represents the most consequential health policy legislation Vermont has passed since Act 48 created the GMCB in 2011.

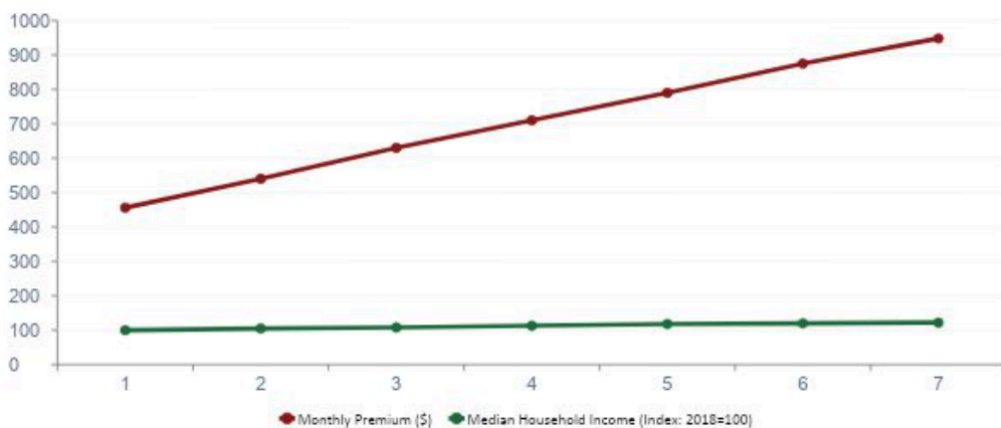


Figure A.7 — Vermont Premium vs. Income Growth 2018–2024: 108% premium increase vs. 22% income growth — the affordability gap driving Vermont's transformation mandate. Source: Claude_HTR_Visual_Companion_Deck.pptx, Slide 12. AHS January 2025 Legislative Presentation; GMCB rate filings.

A.7 Vermont Healthcare Workforce

Vermont's healthcare workforce faces a triple constraint that no single intervention can resolve: insufficient supply of primary care physicians, a specialist shortage that cannot be addressed by adding more specialists to each community, and a housing and economic environment that makes recruiting and retaining clinical staff increasingly difficult.

The primary care shortage is the most immediately actionable. Vermont's RHT Program application projects a shortage of 370 full-time equivalent primary care providers by 2030 — 112 in family medicine and 190 in other primary care specialties. Oliver Wyman's analysis offered a counterintuitive but important qualification: HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is not purely one of physician headcount — it is partly one of primary care productivity and care model. If primary care providers were supported to see three patients per hour in team-based care settings where clinical staff work at the top of their licensure and administrative burden is minimized, Vermont would have adequate primary care supply for well into the future.

The specialist shortage requires a different solution: regionalization. Vermont cannot maintain cardiologists, neurologists, radiation oncologists, and orthopedic surgeons at every one of its 14 hospitals. The patient volume at smaller community hospitals does not justify the positions, and the clinical volume needed to maintain specialist expertise cannot be generated at a hospital serving 30,000–50,000 people. Oliver Wyman's Center of Excellence framework — concentrating specialty services at facilities with sufficient volume — is the correct response. The key is ensuring that concentration does not mean inaccessibility: telehealth consultation, reliable EMS transport, and clear referral pathways are the operational complements to regionalization.

The housing constraint is the most intractable and the most important to name honestly. Vermont's rental vacancy rate of 3% is well below the 5% that characterizes a healthy rental market. Half of Vermont renters are cost-burdened. Vermont has the second-highest per-capita homelessness rate in the country. Healthcare providers recruited to rural Vermont communities face a housing market that may make it impossible to find an affordable place to live near the hospital where they would work. Oliver Wyman's first imperative — build housing and fix transportation — is a healthcare workforce policy as much as a social policy. The transformation plans submitted in January 2026 reflect this: SVMC's plans to reduce locum provider costs, Mt. Ascutney's Nursing Pathways Program funding two cohorts of nursing students at \$190,000, and Grace Cottage's near-elimination of travel nurses through local retention strategies all reflect the same underlying constraint — rural Vermont cannot recruit and retain clinical staff at the volume and cost structure that fee-for-service hospital operations require.

A.8 Vermont Clinical Quality Indicators

Vermont's clinical quality performance reflects both the strengths of a high-coverage, primary care-oriented system and the equity gaps that demographic and geographic disadvantage produces. The Blueprint for Health, Vermont's statewide Patient-Centered Medical Home initiative, generated a 5.8 to 1 return on investment from its primary care investment — among the most robust ROI findings in the value-based care literature — and provides the clinical infrastructure on which the AHEAD Model's primary care redesign builds.

Vermont's 11-point primary care access gap between white and BIPOC residents, the Northeast Kingdom's elevated chronic disease burden, and the rural-urban disparities in behavioral health access are the equity targets that the transformation agenda must measure against. These indicators are tracked in GMCB's annual performance reports and provide the baseline against which AHEAD Model equity requirements will be assessed. See Chapter 10 (The Equity Pillar) for the full equity analytics framework.

Sources

- AHS Hospital Tracking data (January 2026)
- AHS Transformation_Plans_Analysis.xlsx (January 2026)
- Individual hospital transformation plans (January 15–31, 2026)
- GMCB Annual Report 2024
- GMCB FY25 and FY26 Budget Orders
- Oliver Wyman Act 167 Report (September 2024)
- AHS Health Care System Transformation Report (November 2025)
- Vermont RHT Program Application (2025)
- AHEAD State Agreement — Cohort 2 (2025)
- KFF State Health Facts — Vermont

- GMCB Act 68 RBP Update (February 2026)
- Mt. Ascutney Transformational Grant January 15 Report (2026)
- WCAX / Vermont Business Magazine — GMCB FY25 budget coverage
- RAND Hospital Transparency; GMCB FY2026 data